Denial Navigator

Feature guide: every screen, what to click, what to say

Live demo: https://ca-dn-sq5zvttffcgfu.agreeablewater-ff7c59b5.eastus2.azurecontainerapps.io
Explainer: https://ca-dn-sq5zvttffcgfu.agreeablewater-ff7c59b5.eastus2.azurecontainerapps.io/explainer

One page per feature: **Where** to go, what to **Click**, what the audience will **see**, a line to **Say**, and what to do **if it goes wrong**. Screenshots come from a test copy running the same release as the live site.

Prepared by Alif Consulting · All data is synthetic: a fictional hospital (Prairie Ridge), fictional payers and patients. No PHI.

Before you start (5 minutes)

1. Open the live demo in **Chrome or Edge**. Use a **private / incognito window** so the browser can't show an old cached copy.
2. Bottom of the left sidebar: **“AI: Azure OpenAI”** and **“Version …”**. If there is no Version line, the new release is not live yet; use the private window and tell the team.
3. **Remittance inbox → Reset demo data → OK**. Wait about 10 seconds.
4. Check: both live files show a plain **Ingest** button; Dashboard shows **Resolved 0** and Teams alerts “No alerts yet”.
5. Open a second tab with **/explainer**. Zoom 90–100%. Do Not Disturb on.

GOLDEN RULES DURING THE DEMO

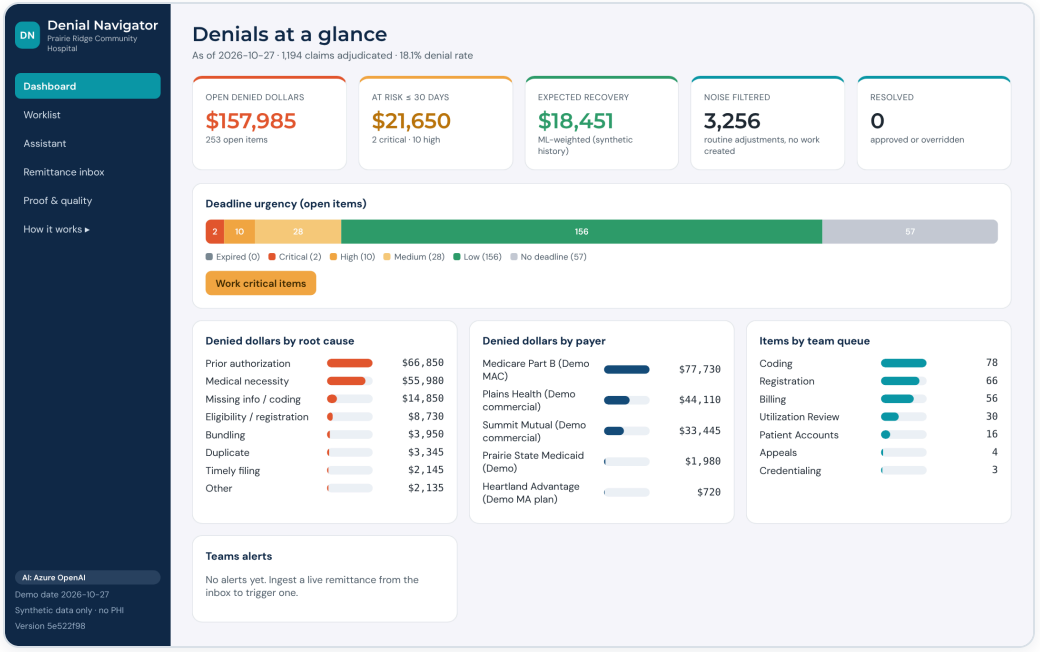
- Click **Ingest once** and watch the seconds counter. Other buttons are locked until it finishes.
- Ingest the **SMM** file first (it fires both Teams alerts), then the Medicare file.
- To repeat anything, use **Reset demo data**, not “Ingest again”.
- If a page looks odd: **Cmd + Shift + R**.
- If the app won't load at all: play the explainer MP4 and walk through these screenshots.

Suggested 10-minute order

Page	Time	Feature
1	0:45	Dashboard
2	0:30	Worklist
3	0:45	Ingest SMM file
4	0:30	Teams alerts
5	1:30	PRH-10222 claim view
6	0:30	Draft letter
7	0:15	Approve
8	1:00	Ingest Medicare → PRH-10224
9	0:45	PRH-10227, PRH-10225
10	0:30	Explain with AI
11	0:45	Override
12	0:45	Assistant
14	0:30	Proof & quality
15	1:00	Optional opener: the explainer video
16	—	Reset demo data before and after (not shown)

Pages 13 (upload) and 15 (explainer) are optional depending on time. Close with the deck's data-trust and next-steps slides.

01 Dashboard



WHERE

Left menu → **Dashboard** (the page that opens first).

CLICK

1. Nothing to click. Point at the five tiles left to right, then the coloured urgency bar, then the four charts.

YOU'LL SEE

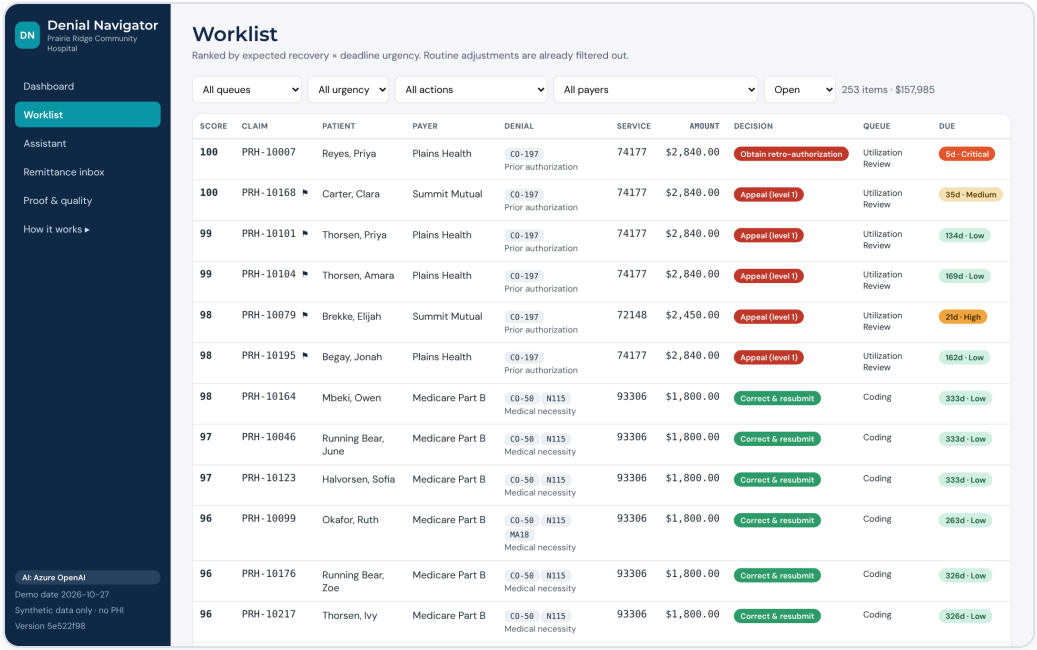
- **Open denied dollars** ($157,985 across 253 items), **At risk ≤ 30 days**, **Expected recovery**, **Noise filtered** (3,256), **Resolved** (0 at the start).
- Urgency bar: how many open items are Critical, High, Medium, Low or have no deadline.
- Charts: denied dollars by root cause and by payer, items by team queue, and the Teams alerts card.

> ### SAY
>
> "This is a 25-bed rural hospital: about 1,200 claims and an 18% denial rate. More than 3,000 routine adjustments were filtered out automatically, so the team only sees the 253 denials that need work, with the money and the deadlines up front."

IF IT GOES WRONG

Page blank or old-looking: press Cmd + Shift + R. Resolved is not 0: go to Remittance inbox → Reset demo data.

02 Worklist



WHERE

Left menu → **Worklist**.

CLICK

1. Point at the **Score** column (highest first).
2. Set **Queue** to *Coding* to show one team's view, then back to *All queues*.
3. Optional: **Urgency** = *Critical*, or click **Work critical items** on the Dashboard.

YOU'LL SEE

- Every denial with patient, payer, denial code, service, amount, the decided action, the team queue and the days left.
- Rows are ranked by expected recovery × deadline urgency. Filters change the list instantly; the count and dollar total update above the table.

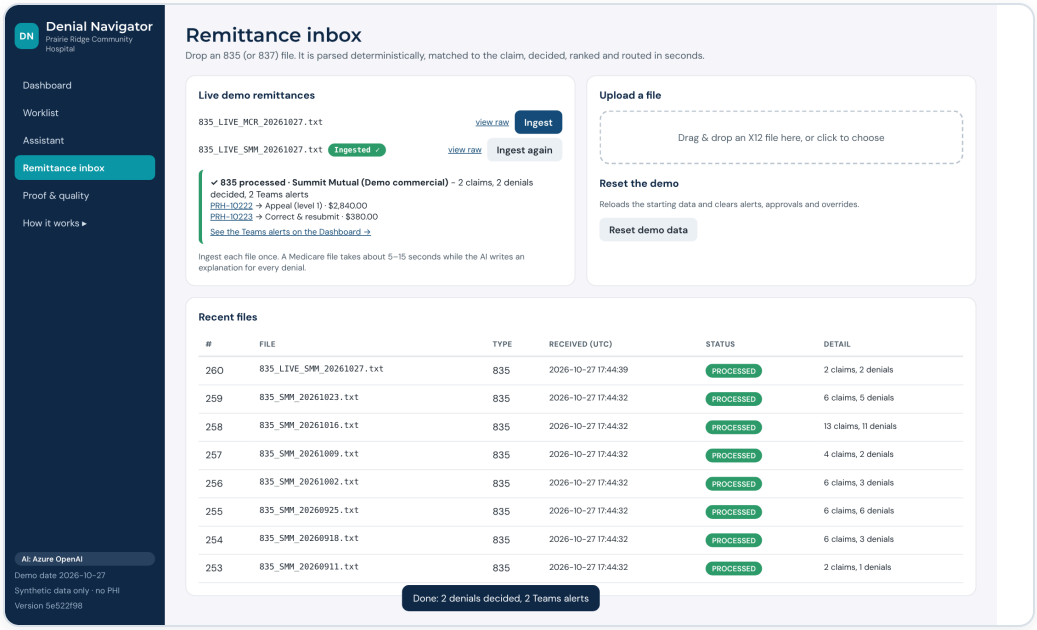
> **SAY**
>
> "Nobody sorts a spreadsheet any more. Each team opens its own queue, and the most valuable and most urgent items are at the top."

IF IT GOES WRONG

List looks unchanged after a filter: wait a second for "Loading…" to finish. Click any row to open the claim.

03 Remittance inbox: a payer file arrives





WHERE

Left menu → **Remittance inbox**.

CLICK

1. Click **Ingest** on **835_LIVE_SMM_…** once.
2. Watch the button count *Processing… 1s, 2s*. The other buttons are locked until it finishes.

YOU'LL SEE

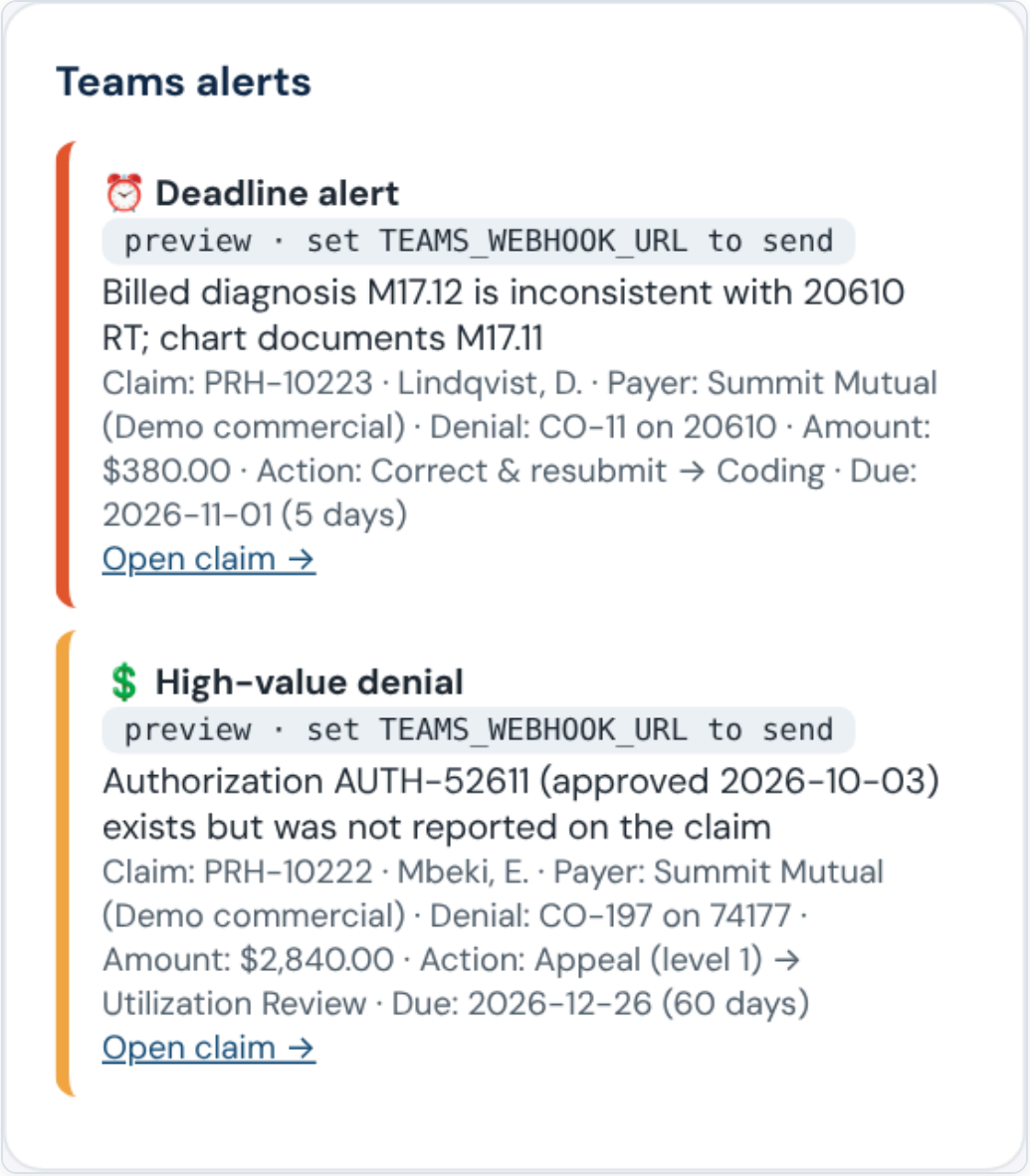
- Within about 5 seconds: **"✓ 835 processed · Summit Mutual – 2 claims, 2 denials decided, 2 Teams alerts"**.
- PRH-10222 → Appeal (level 1) · $2,840 and PRH-10223 → Correct & resubmit · $380.
- The file gets an **Ingested ✓** badge and appears at the top of Recent files.

> **SAY**
>
> "A payer file just arrived. Nobody has to read it: it's parsed, matched to the claims, every denial is decided and routed, and the team is alerted, in seconds."

IF IT GOES WRONG

Red "Could not process" box: wait 5 seconds and click Ingest once more. File ingested twice by mistake: Reset demo data and start again.

04 Teams alerts



WHERE

Click **See the Teams alerts on the Dashboard →** in the result, or left menu → **Dashboard**.

CLICK

1. Point at the two alert cards.
2. Click **Open claim →** on the PRH-10222 card.

YOU'LL SEE

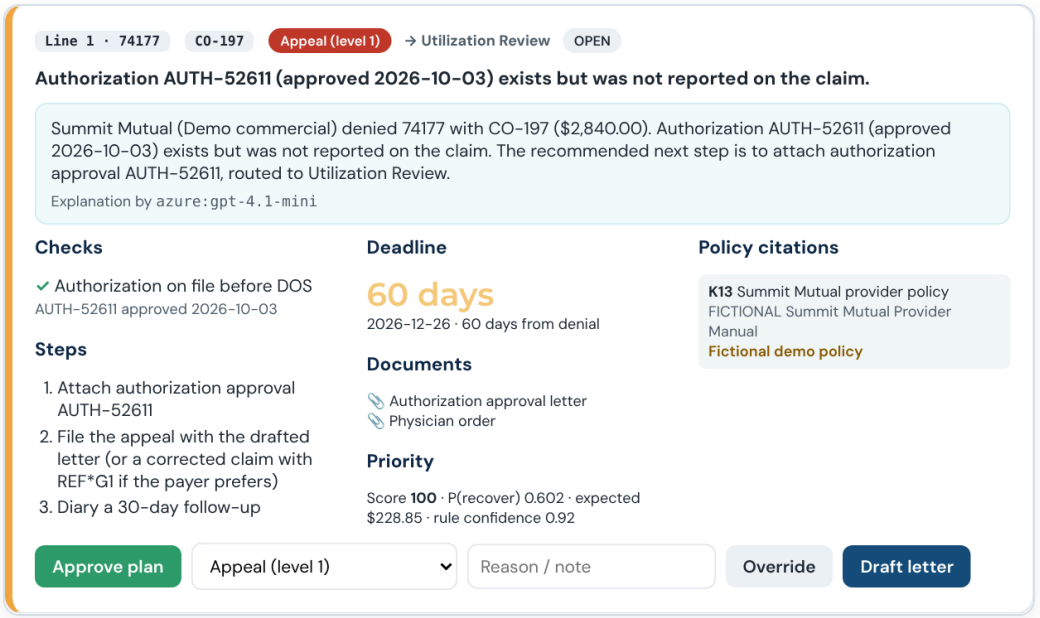
- 💲 **High-value denial**: PRH-10222, $2,840 prior authorization.
- ⏰ **Deadline alert**: PRH-10223, a coding fix due in 5 days.
- Each card is the exact Adaptive Card that is posted to the team's Teams channel (it shows “preview” because no channel is connected in the demo).

> **SAY**
>
> “The right team is told in Teams: one alert because the dollars are high, one because a deadline is about to be missed.”

IF IT GOES WRONG

No alerts: the SMM file was not ingested yet. Go back to step 3.

05 Claim view: decision, evidence, deadline



WHERE

Open claim → on the alert, or Worklist → click **PRH-10222**.

CLICK

1. Read the bold cause line.
2. Point at **Checks**, **Steps**, **Deadline**, **Documents**, **Priority** and **Policy citations**.
3. Scroll down to show the claim lines (837) next to what the payer paid (835) and the EHR facts used.

YOU'LL SEE

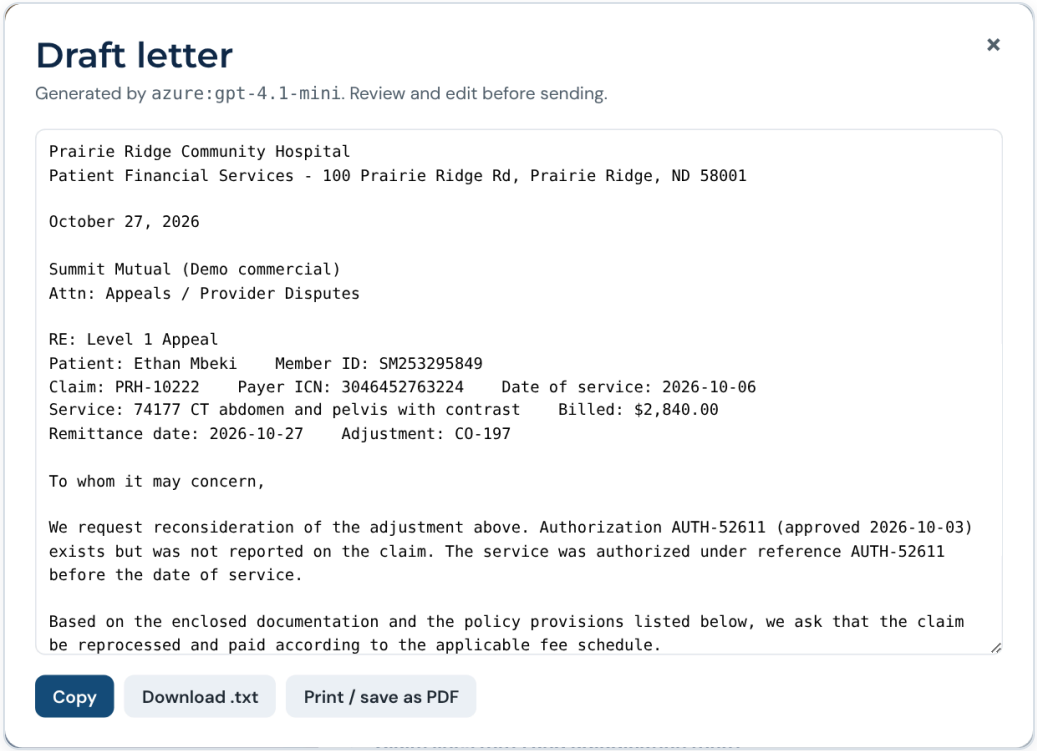
- Decision **Appeal (level 1) → Utilization Review**.
- Cause: authorization AUTH-52611 was approved before the visit but not reported on the claim.
- Plain-English explanation written by Azure OpenAI, a policy citation, and a 60-day countdown.

> ### SAY
> "$2,840 prior-auth denial. It found the authorization was on file but missing from the claim, so the answer is appeal, not write-off. It cites the payer policy and computes the deadline. Rules make the decision; the AI explains it."

IF IT GOES WRONG

Drawer shows "Could not load": click Try again. Close the drawer with × or Esc.

06 Draft the appeal letter



WHERE

Inside the PRH-10222 claim view.

CLICK

1. Click **Draft letter** (about 4 seconds).
2. Scroll the letter. Optional: **Copy**, **Download .txt**, or **Print / save as PDF**.
3. Close with ×.

YOU'LL SEE

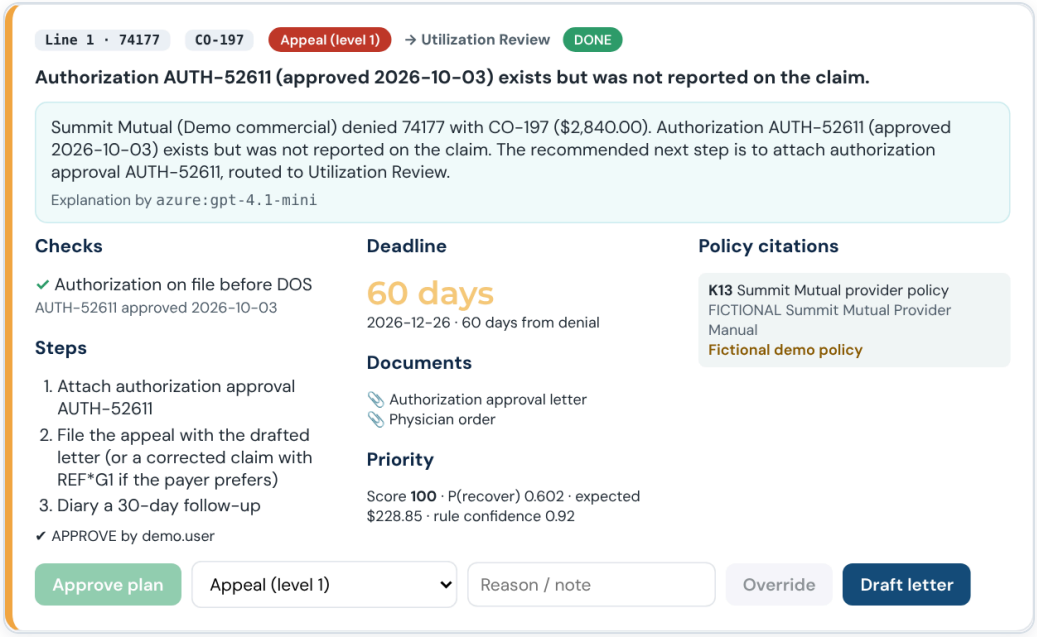
- A complete appeal letter with patient, member ID, claim number, payer ICN, service, the authorization number, the enclosures and the policy references.
- It is editable: the person reviews it before it is sent.

> **SAY**
>
> "The appeal letter is drafted with every number already filled in. A person reviews and edits it; nothing is sent on its own."

IF IT GOES WRONG

"Could not draft the letter": close it and click Draft letter again.

07 Approve the plan



WHERE

Inside the claim view, after the letter.

CLICK

1. Click **Approve plan**.

YOU'LL SEE

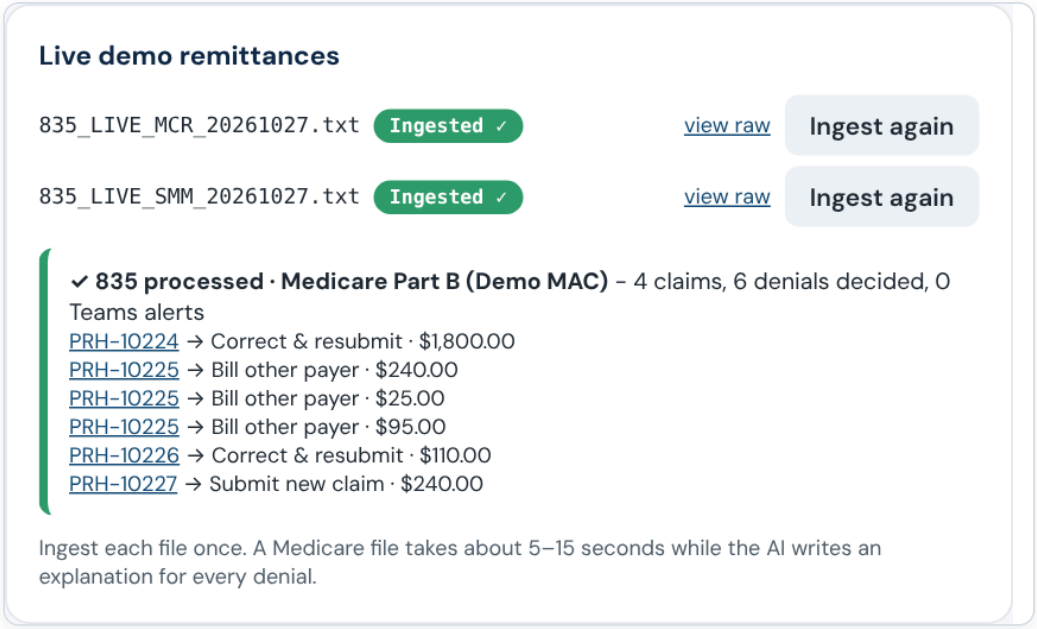
- Toast: “Plan approved – item moved to resolved”. The status badge changes to **DONE** and the approval is recorded with the user.
- Dashboard → **Resolved** goes up by 1.

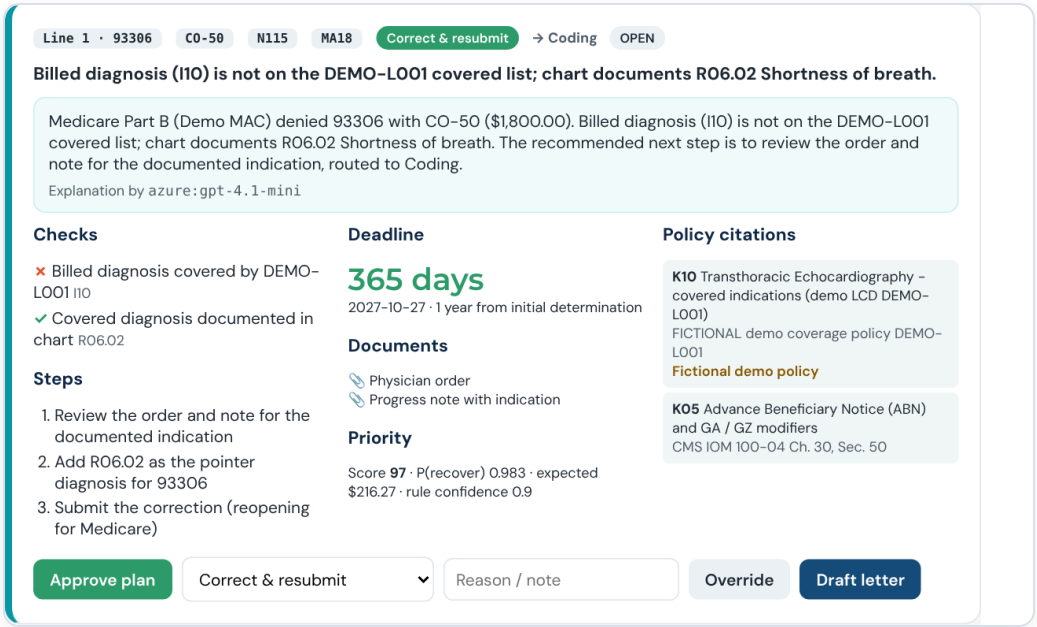
> **SAY**
>
> “A person approves every appeal and write-off. Every decision and approval is logged.”

IF IT GOES WRONG

Button greyed out: the item is already approved. Pick another open item.

08 Medicare file: medical-necessity denial





WHERE

Left menu → **Remittance inbox**.

CLICK

1. Click **Ingest** on **835_LIVE_MCR_…** once (about 5 seconds).
2. Click **PRH-10224** in the result.

YOU'LL SEE

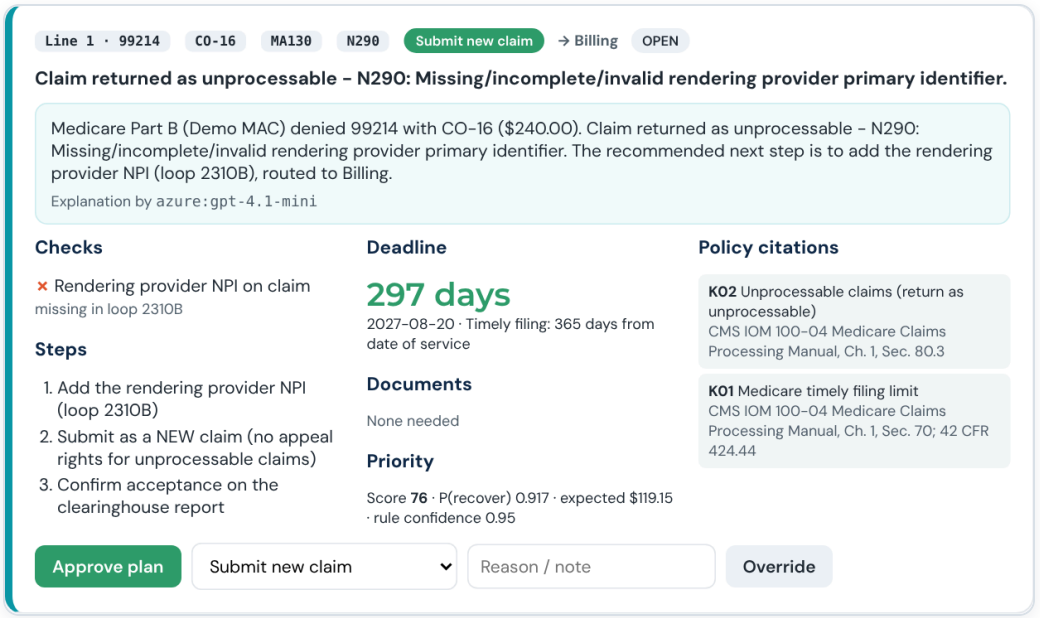
- "4 claims, 6 denials decided".
- PRH-10224 (echocardiogram, $1,800): **Correct & resubmit → Coding**.
- Checks: × the billed diagnosis (I10) is not on the coverage list; ✓ a covered diagnosis (R06.02) is documented in the chart. Deadline: reopening within 1 year.

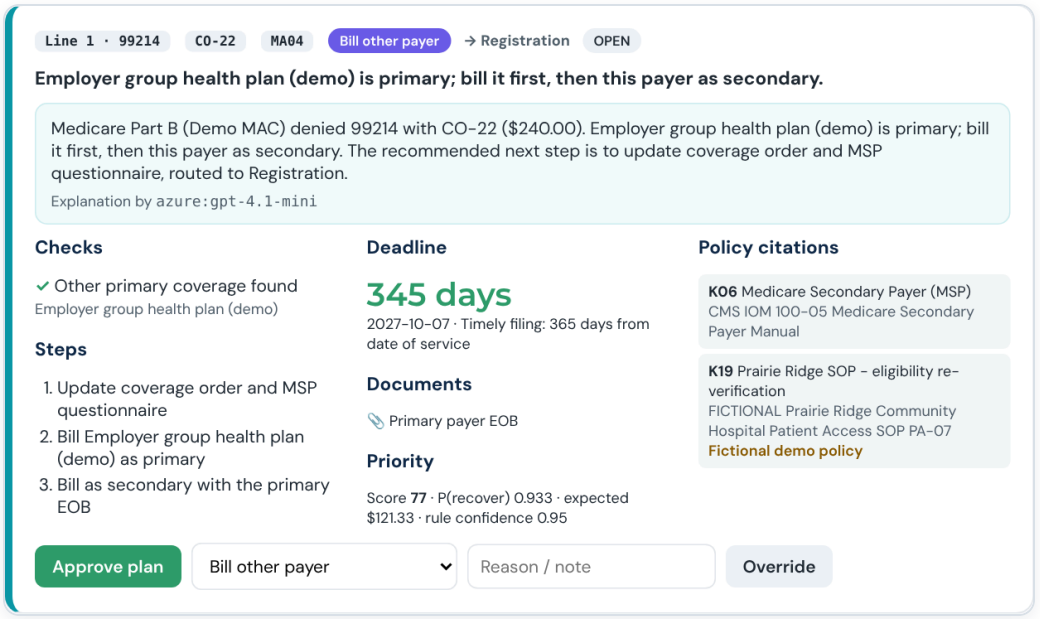
> **SAY**
>
> "Medical necessity on an echocardiogram. The billed diagnosis isn't covered, but a covered one is in the chart. So the fix is to correct and resubmit, not write $1,800 off."

IF IT GOES WRONG

While it runs, say: "the AI is writing an explanation for each Medicare denial, all at once."

09 Three more denial types





WHERE

Open each claim from the Medicare result or the Worklist.

CLICK

1. **PRH-10227**
2. **PRH-10225**
3. **PRH-10226** (optional)

YOU'LL SEE

- PRH-10227: rendering provider NPI missing (CO-16 MA130) → **Submit new claim → Billing**. No appeal: unprocessable claims have no appeal rights.
- PRH-10225: employer plan is primary (CO-22) → **Bill other payer → Registration**, all three lines.
- PRH-10226: bundling edit (CO-97) where a modifier is allowed and a separate session is documented → **Correct & resubmit → Coding**.

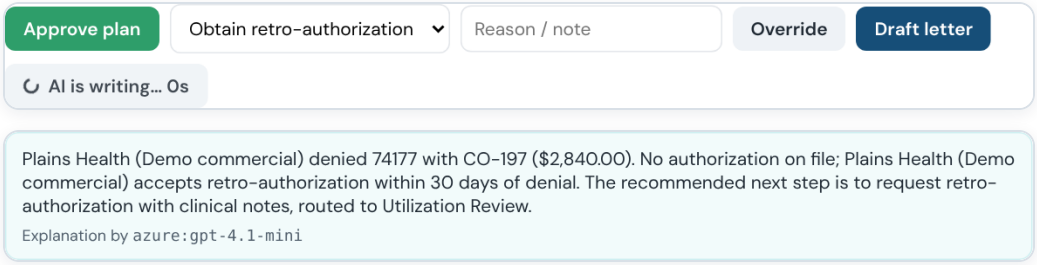
SAY

"Different denials, different fixes, different teams. Here it saves a wasted appeal; here it sends the bill to the right payer first."

IF IT GOES WRONG

—

10 Explain with AI



WHERE

Worklist → open any item that is *not* from the two live files (for example the top row).

CLICK

1. Click **Explain with AI**. The button shows *AI is writing… 1s, 2s*.

YOU'LL SEE

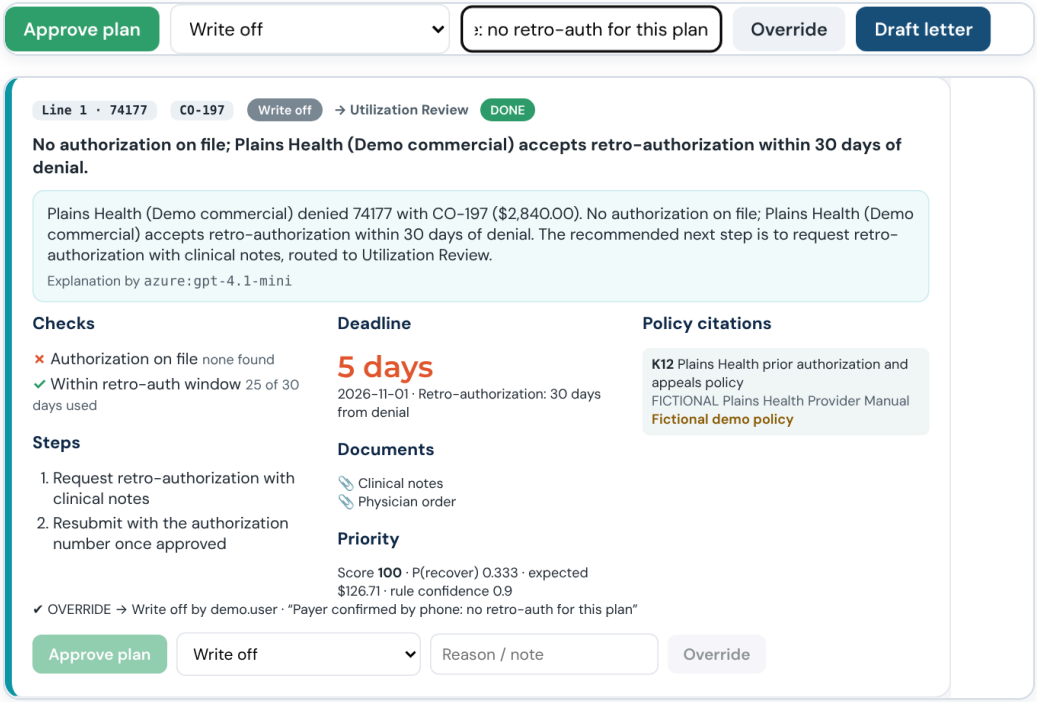
- A plain-English explanation appears, marked **Explanation by azure:gpt-4.1-mini**.
- The action, team and deadline do not change: the AI explains, the rules decide.

> ### SAY
>
> “Older items were explained by templates. One click and Azure OpenAI writes a plain-English explanation, but it can't change the decision.”

IF IT GOES WRONG

Button missing: that item already has an AI explanation. Pick another row.

11 Human override becomes a precedent



WHERE

Any open item in the claim view.

CLICK

1. Pick a different action in the drop-down.
2. Type a reason (Override without a reason asks for one).
3. Click **Override**.

YOU'LL SEE

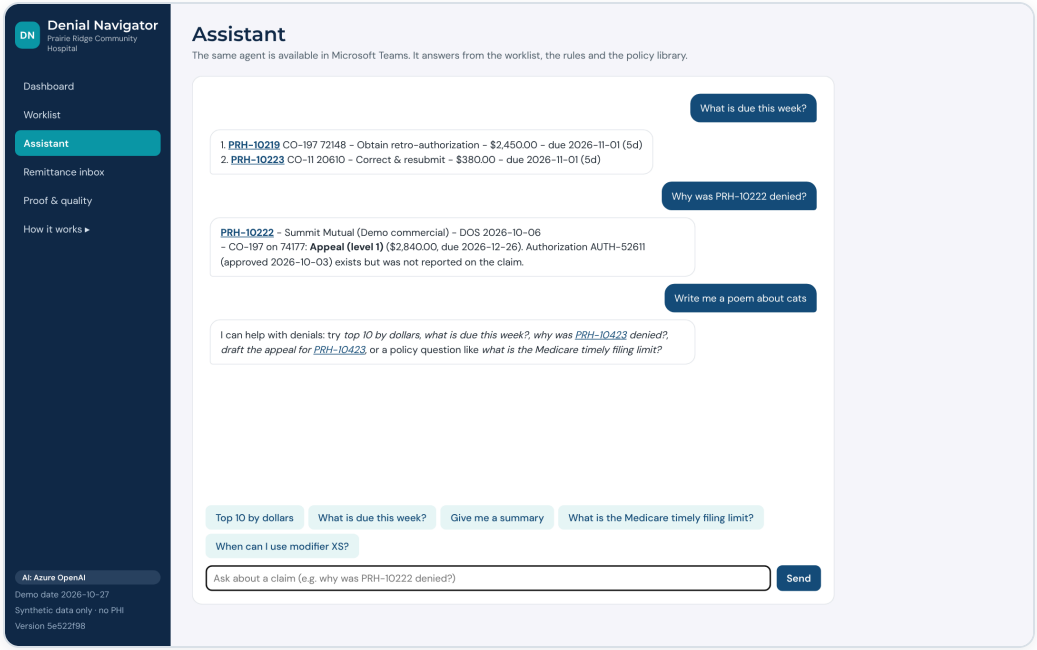
- Status **DONE**, with "OVERRIDE → … by demo.user · 'reason'" recorded under the card.
- The override is added to the policy library as a precedent, so similar future cases cite it. Resolved goes up by 1.

> **SAY**
>
> "People stay in control. When an expert disagrees, the override and the reason become a precedent the system learns from."

IF IT GOES WRONG

Nothing happens: type a reason first.

12 Assistant (same as in Teams)



WHERE

Left menu → **Assistant**.

CLICK

1. Click **What is due this week?**
2. Type **Why was PRH-10222 denied?** and press Send.
3. Type **Write me a poem about cats**.

YOU'LL SEE

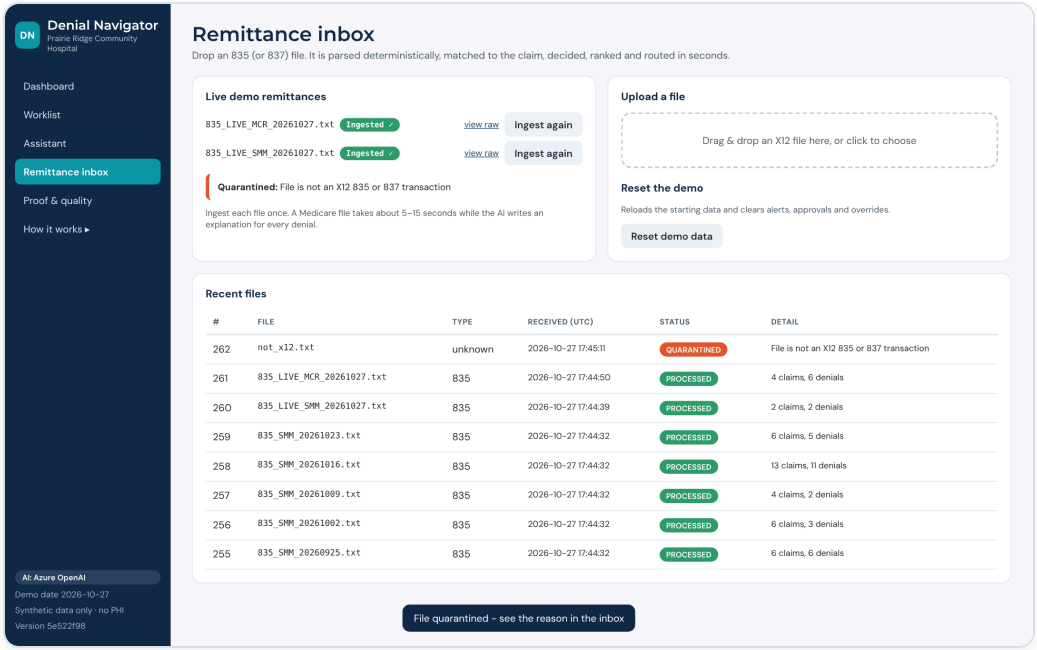
- The due list with claim numbers, dollars and dates; a claim explanation; and a polite refusal to go off topic.
- Claim numbers in answers are links that open the claim. The Send button is locked until each answer arrives.
- Screenshot from the test copy: on the live site Azure OpenAI writes the answers, so the wording differs.

> **SAY**
>
> “Managers just ask. The same assistant works in Microsoft Teams, and it stays on topic.”

IF IT GOES WRONG

Slow answer (up to ~8 s): keep talking; it shows “Thinking…”.

13 Upload a file; bad files are quarantined



WHERE

Left menu → **Remittance inbox** → **Upload a file**.

CLICK

1. Drag any text file onto the box, or click it and choose a file.

YOU'LL SEE

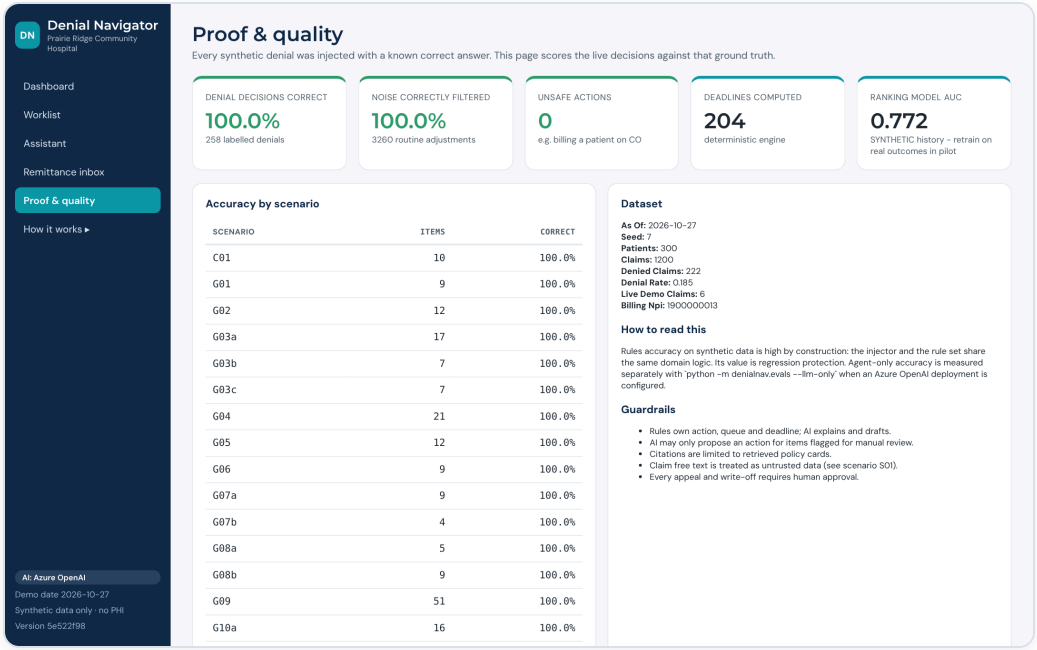
- A non-X12 file is **Quarantined** with the reason; nothing breaks.
- A real 835 or 837 file is processed exactly like the live files.
- **view raw** next to a live file shows the original X12 text.

> **SAY**
>
> “Bad files don't break anything: they're quarantined with a reason.”

IF IT GOES WRONG

Optional step; skip it if short of time.

14 Proof & quality



WHERE

Left menu → **Proof & quality**.

CLICK

1. Point at the five tiles, then Accuracy by scenario and Guardrails.

YOU'LL SEE

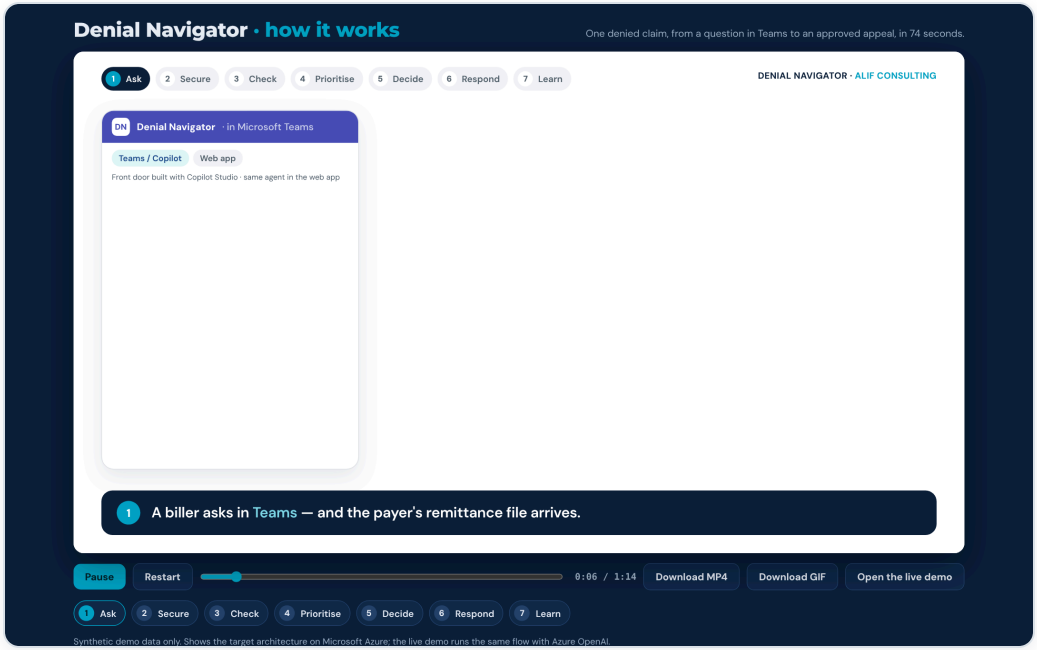
- **Denial decisions correct 100%** (258 labelled denials), **noise filtered 100%**, **0 unsafe actions**, deadlines computed, ranking AUC 0.772.
- Guardrails: rules own the action, AI explains; citations only from the policy library; every appeal and write-off needs a person.

> ### SAY
>
> “Every synthetic denial was injected with a known right answer, and this page scores the live decisions against it: zero unsafe actions. The pilot measures the same thing on your real data.”

IF IT GOES WRONG

—

15 How it works (explainer, GIF, MP4)



WHERE

Left menu → **How it works ▸** (opens a new tab), or https://ca-dn-sq5zvttffcgfu.agreeablewater-ff7c59b5.eastus2.azurecontainerapps.io/explainer

CLICK

1. Press Play, or jump with the 7 step buttons.
2. **Download MP4** for the deck, **Download GIF** for email or Teams.

YOU'LL SEE

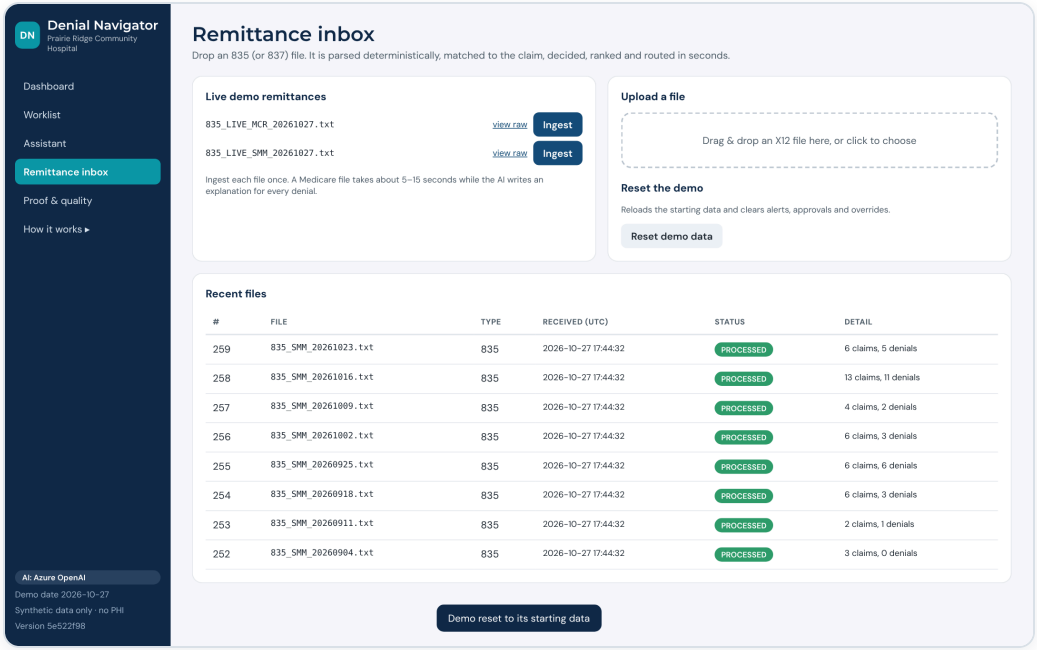
- The 74-second story: Ask, Secure, Check, Prioritise, Decide, Respond, Learn.

> **SAY**
>
> "Here's the whole idea in one minute: one denied claim, from a question in Teams to an approved appeal."

IF IT GOES WRONG

—

16 Reset demo data



WHERE

Left menu → **Remittance inbox** → **Reset demo data** (right-hand card).

CLICK

1. Click **Reset demo data** → **OK** (about 10 seconds).

YOU'LL SEE

- Both live files show a plain **Ingest** button again, Recent files has no LIVE files, Dashboard shows Resolved 0 and “No alerts yet”.

> **SAY**
>
> (Not shown to the audience. Do it before and after every demo.)

IF IT GOES WRONG

App won't open at all: in Cloud Shell, restart the revision (see the runbook), wait 2 minutes.

? Likely questions

Question	Answer
Does our data train the AI?	No. It is processed in your Azure tenant, and Azure OpenAI does not use your prompts or data to train its models. It learns your patterns only from your own resolved claims, inside your tenant.
How accurate is it?	Codes, edits and deadlines are decided by rules, so they are exact. Against known answers: 100% correct, zero unsafe actions. The pilot measures accuracy on your real outcomes.
What does the AI actually do?	It explains decisions in plain English, drafts letters and answers questions. It cannot change an action, a queue or a deadline.
What does it cost to run?	About $200 a month on Azure for a small hospital. One recovered denial a month covers it.
Does it need EHR integration?	Not to start. It reads standard 835 and 837 files. Clearinghouse and EHR links come in the scale phase.
Is sign-in supported?	Yes. Entra ID single sign-on is a switch; it is off for this demo only.
Is this real patient data?	No. Everything is synthetic.